Selective Spinal Immobilization

TB-08 | Trauma & Burns | Reviewed

Clinical release pending. Verify current approved protocols and standing orders.

Quick flow

01 Blunt Trauma [EMT / AEMT / Paramedic]

Manually stabilize when spinal injury is reasonably suspected.

02 Reliable Exam? [EMT / AEMT / Paramedic]

Normal alertness • no intoxication • no distracting injury • effective communication.

03 SMR Indicated? [EMT / AEMT / Paramedic]

Altered mental status • midline neck/back pain or tenderness • focal neurologic symptom/deficit • spinal deformity • unreliable examination.

04 SMR Indicated [EMT / AEMT / Paramedic]

Use an appropriately fitted cervical collar and keep head, neck, and torso aligned and secured on the cot, scoop, vacuum device, or similar device.

05 SMR Not Indicated [EMT / AEMT / Paramedic]

Mechanism alone does not mandate SMR in an alert, reliable patient with no pain/tenderness or neurologic finding. Document the negative assessment and reassess if symptoms change.

06 Transport + Reassess [EMT / AEMT / Paramedic]

Use the safest position that preserves airway and ventilation; repeat motor and sensory examinations and document findings at handoff.

Overview

- Apply spinal motion restriction (SMR) selectively after blunt trauma based on a reliable clinical examination and high-risk findings.
- The goal is to minimize unwanted spinal movement and provide safe handling, not routine prolonged placement on a rigid long board.

Provider-Level Actions

Perform all immediately indicated care within scope without delaying rapid trauma transport.

EMT

- Assess alertness, reliability, midline neck/back pain or tenderness, neurologic findings, deformity, distracting injury, intoxication, and communication barriers.
- For children, include neck pain/torticollis, neurologic abnormality, altered mental status, high-risk motor-vehicle collision, high-impact diving, and substantial torso injury in the age-appropriate assessment. Age alone is not an indication, and mechanism alone does not mandate SMR in an alert, reliable, asymptomatic patient.
- Use appropriately sized equipment and padding to avoid excessive neck flexion; maintain airway access, do not force neutral alignment, and minimize time on rigid transfer devices.
- Apply SMR when indicated using an appropriately fitted cervical collar and aligned securement; pad voids, protect pressure points, and reassess motor/sensory findings and distal perfusion before and after movement.

AEMT

- Perform all EMT care and support safe positioning, airway care, analgesia, and serial neurologic reassessment within scope.

- Expanded AEMT analgesia is not enabled by default while individual authorization is unverified. The specified medication, route, and patient group require current Tennessee authority, documented Tennessee-approved expanded analgesia training, agency credentialing, and medical-director standing orders; training alone does not grant permission.

Paramedic

- Perform all prior care, resolve equivocal examinations, and manage airway or ventilation while minimizing cervical motion.
- Document the clinical basis for applying or clearing SMR and repeat neurologic findings before transfer of care.

Indications

- Patient with suspected selective spinal immobilization requiring prehospital assessment or treatment.

Warnings / precautions

- Do not force the head or neck into neutral alignment when pain, resistance, neurologic change, airway compromise, or a position that cannot be tolerated occurs.
- A rigid long board is primarily an extrication or transfer device, not routine transport packaging; remove it when safely feasible.
- Routine SMR is not indicated for penetrating trauma without neurologic deficit and should not delay transport.
- For pediatric clearance, use only the approved pediatric decision aid and locally authorized clinician. If these requirements cannot be met, obtain Medical Control guidance; do not substitute an emergency-department imaging rule or treat these risk features as a stand-alone field-clearance rule.

Source and references

Claiborne County EMS native protocol content; controlled PDF generated from the same structured source. | Current Tennessee EMS scope of practice, verified individual credentials, and Claiborne County standing orders control provider authorization.

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